Question:

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

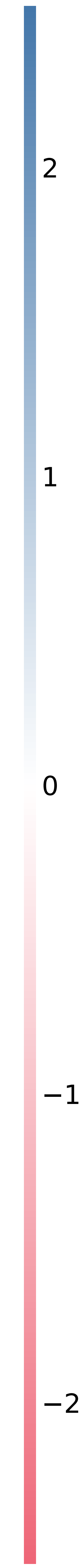
A. Neonatal respiratory distress syndrome (NRDS)

B. Tracheoesophageal fistula

C. Pneumonia

D. Congenital heart anomaly with right-to-left shunt

Reasoning + Answer:



<think>Reasoning process here:

1. **Clinical Presentation**: The symptoms presented (tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis) are indicative of a respiratory problem, particularly in a neonate.
2. **Gestational Age**: The baby was born at 32 weeks gestation, which places it at high risk for various neonatal complications due to prematurity.
3. **Diabetes of the Mother**: Gestational diabetes increases the risk of hypoglycemia in the neonate, but this isn't the primary issue here.
4. **Oxygen Saturation**: Low oxygen saturation at birth requires immediate attention, increasing the suspicion of a respiratory or cardiovascular issue.
5. **Response to Oxygen**: Improvement in oxygen saturation with administration of oxygen suggests a potential reversible cause.
6. **Positioning of the Nasogastric Tube**: There's no indication that the tube's placement affected the diagnosis.

Given these considerations, we need to weigh the likelihood of each condition:
- **Neonatal Respiratory Distress Syndrome (NRDS)** is a plausible cause, given the infant's prematurity and respiratory symptoms.
- **Tracheoesophageal Fistula** is typically identified by feeding difficulties and doesn't align well with the clinical presentation provided.
- **Pneumonia** might present with similar respiratory symptoms but would likely have different risk factors and lung auscultation findings not mentioned.
- **Congenital Heart Anomaly with Right-to-Left Shunt** could present with cyanosis but usually includes cardiac-specific auscultation findings not described here.

<answer>D. Congenital Heart Anomaly with right-to-left shunt</answer>